

Public-Sector Procurement Intelligence — Webinar Notes

Dean's notes from a procurement-intelligence webinar (a market-data platform for spotting public-sector contracts, buyer behaviour and pre-tender engagement) — read for what it means for OTOS's route to Cambridge/public-sector funding, not as a reason to chase tenders.

Draft status: founder notes from a live webinar, 2026-07-22. Not procurement advice. Figures are as presented by the speaker/platform — verify before external use.

WHY THIS WEBINAR MATTERS FOR OTOS

The session was about using public-sector procurement intelligence to spot contracts, understand buyer behaviour, and engage before tenders go live. The useful takeaway for OTOS is **not** "bid for everything" — it is to use market data to choose the right public-sector route, partners, buyers, and timing. It also independently confirms the direction the Cambridge First-50 plan is already on: relationships and evidence before any formal procurement route.

1 · Public sector is huge, but frameworks dominate value

FIGURE (AS PRESENTED)	WHAT IT MEANS FOR OTOS
UK public-sector spend ≈ £440bn/year .	The addressable space is real, but scale isn't the near-term target.
~ 72% of total contract value runs through frameworks — frameworks are only ~ 28% of contract volume.	Frameworks carry most of the money even though most contracts by count are outside them. OTOS shouldn't ignore frameworks, even though the Cambridge demonstrator

FIGURE (AS PRESENTED)**WHAT IT MEANS FOR OTOS**

starts through local relationships, pilots, grants, or innovation routes.

OTOS consideration: map which frameworks or routes might eventually fit OTOS Continuity™ — digital health, population health, prevention, social care, VCSE, transformation, mental health, addiction, ADHD, and NHS innovation routes.

2 • The median award value is realistic for a supplier OTOS's size

Median individual award value cited: **£132,000**. Headline framework totals can mislead because large frameworks distort the average — smaller suppliers and SMEs still win standalone or locally-focused contracts.

The demonstrator's current rough funding envelope — around **£150k** — sits close to this median. That supports positioning OTOS as a realistic early-stage public-sector supplier rather than assuming everything needs a huge national contract.

3 • Pre-market engagement is the main advantage

The strongest repeated message: if a supplier waits until the tender is published, they may already be too late. Buyers shape specifications *before* tender. Supplier conversations should happen before the tender goes live — engagement during a live tender is restricted and can raise collusion concerns.

OTOS consideration: the important work is early relationship-building with the people who shape need and service design, not just procurement contacts. This fits the current Cambridge approach exactly — Public Health, NHS leads, addiction services, ADHD/mental-health stakeholders, VCSE partners, commissioners, and innovation contacts, before any formal procurement route exists.

4 • Speak to decision-makers, not only procurement

Procurement teams often run the process but don't always define the need. Useful target roles: senior leaders, commercial leads, digital leads, sustainability/social-value leads, service leads, and people responsible for outcomes and transformation.

ROLE TYPE	CAMBRIDGE EQUIVALENT
Prevention / population health	Public Health / prevention leads
Commissioning	Mental health and addiction commissioners
Clinical pathway	ADHD / neurodevelopmental pathway leads
Transformation	ICB transformation or innovation leads
VCSE / community	VCSE / recovery service leads
Acute / discharge	Hospital alcohol liaison / discharge / continuity stakeholders
Digital	Digital transformation leads, where appropriate

OTOS consideration: identify who owns the pain, not just who buys systems.

5 • Social value and sustainability now score more heavily

The Procurement Act 2023 shifts evaluation from "Most Economically Advantageous Tender" to "**Most Advantageous Tender**" — more weight on social value, sustainability, local impact, and wider benefits beyond price.

OTOS should make the social-value case explicit: reducing avoidable relapse and crisis loops; supporting continuity for people who fall between services; improving engagement and handoff reliability; helping local systems use existing services better; reducing wasted appointments, missed follow-up, and repeated re-entry; creating a replicable prevention/continuity model for Cambridge first, then wider areas.

6 • Market intelligence can reveal expiring contracts and entry windows

The platform shown tracks expiring contracts, framework renewals, existing suppliers, buyer activity, spend patterns, relationship strength, and upcoming opportunities.

OTOS consideration: use this kind of intelligence to identify which councils, ICBs, NHS trusts, or public-health bodies already buy relevant digital/prevention/addiction/mental-health/VCSE services; when existing contracts expire; which suppliers currently hold related contracts; where there's a gap OTOS can fill; and whether to partner with an incumbent rather than compete directly.

7 • Frameworks can be used for partner strategy

Being on a framework doesn't guarantee equal revenue — frameworks reveal which suppliers are actually winning work.

OTOS may not need to be the prime supplier at first. A

practical route: partner with an existing framework supplier, add OTOS Continuity™ as a continuity/engagement layer, use the Cambridge demonstrator as evidence, and position OTOS as a specialist non-clinical layer — not a replacement for clinical systems.

8 • Possible OTOS actions from this

NEAR-TERM

- Build a simple list of target public-sector buyer types:
Cambridgeshire County Council / Public Health; Cambridge City Council where relevant; Cambridgeshire & Peterborough ICB; NHS Trust / mental-health / addiction pathway stakeholders; VCSE and recovery partners; innovation and prevention leads.
- For each target, identify: who owns the problem, who can sponsor a demonstrator, who controls or influences funding, who shapes specifications, and what existing contracts or services OTOS could complement.

BEFORE PROCUREMENT

- Do not wait for a tender.
- Use the First-50 demonstrator as a pre-market engagement asset.
- Ask buyers and partners what they would need to see for OTOS Continuity™ to be fundable, commissionable, or partnerable.
- Capture their language and requirements before writing any formal bid.

EVIDENCE TO PREPARE

- Clear non-clinical scope
- Safety and governance boundaries
- Data protection position
- Human review model
- Continuity / handoff receipt explanation
- Pilot costs and expected benefits
- Social value case
- Local system benefit
- Partner roles
- What success looks like after 12 weeks

9 • Key risks / things to be careful about

Do not position OTOS as clinical decision-making software. Keep the language non-clinical: continuity, prompts, handoff visibility, human review.

Do not over-focus on procurement too early. First relationship, evidence, and partner fit — then procurement route.

Do not assume one buyer owns the whole problem. OTOS crosses addiction, ADHD, mental health, public health, VCSE, NHS, and local-authority boundaries.

Avoid waiting for tenders. The webinar strongly suggests winning work happens before the notice goes live.

Be careful with live tenders — once live, conversations are restricted. Pre-tender engagement is the safer, more useful window.

SIMPLE OTOS TAKEAWAY

Use the Cambridge First-50 demonstrator to create early buyer conversations, shape need before procurement, prove social value, and identify whether OTOS should enter public-sector routes directly, through a partner, or through an existing framework supplier. Treat procurement intelligence as a way to choose the right conversations early — not as a reason to chase every tender.

Cross-reference: see also the earlier NHS Procurement Route Response Pack report (HCI/P4H webinar) — that one is NHS-specific; this one is the wider public-sector/framework picture. Read together.

End of report · Public-Sector Procurement Intelligence — webinar notes,
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Internal working document. Source: Dean's notes and recording from a public-sector procurement-intelligence webinar. Verify all figures (spend totals, award medians, framework percentages) before external use.